



PROTOCOL

Complex Attention and Hyperactivity Disorders Service Entry Protocol

Scope (Staff):	Clinical Staff
Scope (Area):	Complex Attention and Hyperactivity Disorder Service (CAHDS)

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#)

Aim

This protocol describes the entry process to Complex Attention and Hyperactivity Disorders Service (CAHDS) including information about eligibility and facilitating access to alternative care for referrals that are not accepted by the service.

Risk

A child may not gain entry to a service that meets their needs in a timely manner.

Background

CAHDS is a state-wide service that works with children, young people (under 18 years old) and families who have persistent difficulties with attention and behaviour. CAHDS provides assessment, diagnosis, support and recommendations to other services, in partnership with the referrer. Children who use the service must have a diagnosis of Attention Deficit Hyperactivity Disorder (ADHD) and are currently being treated for difficulties with paying attention, being hyperactive or controlling impulses.

CAHDS works closely with families, referrers and school staff to provide information in order to improve a client's functioning at home, school and in the community.

CAHDS is informed by recovery oriented practice, and is committed to the following:

- Providing a holistic framework that informs all contact with children, young people and their families.

- Builds and enhances strength, resilience and social and emotional wellbeing. Supports children and young people to engage in meaningful and satisfying life experiences despite their difficulties with ADHD.
- Is underpinned by the premise that children and young people do recover from mental health problems.
- Engages with all areas of the child, young person, and family's life including relationships, education, vocation and leisure.

Key points

For children and adolescents to be eligible for the service, they must be:

- Under the age of 18 years and diagnosed to have ADHD.
- Currently prescribed /being considered for the prescription of medication to treat ADHD.
- Have significant complexity and functional impairment in one or more areas.
- Referred by a Paediatrician, Psychiatrist, Neurologist or an authorised stimulant prescriber who has been providing ongoing treatment for ADHD or related symptoms for the child or adolescent, and who agrees to remain involved with the management of the referred child or adolescent in a shared care management model.

Process

Steps	Additional Information
1. Referral management process Referrals can be made by Paediatricians, Psychiatrists, Neurologists, and authorised stimulant prescribers. Referrers who do not meet this criterion (e.g. allied health clinicians, School Psychologists and General Practitioners) must have their referral accompanied by a supporting letter from the patient's authorised stimulant prescriber agreeing with the referral reasons, goals and to remain the prescriber during the course of CAHDS involvement. All referrals must be completed using the CAHDS Referral Form. Referrals	The CAHDS Clinical Nurse Specialist (CNS) Triage is available to discuss potential referrals. Referrers must indicate on the referral form that they agree to continue managing the care of the client during CAHDS involvement, to be available for consultation with CAHDS Clinicians and to provide follow-up post discharge. Once received, referrals are listed on Psychiatric Services Online Information System (PSOLIS). CAHDS Referral Form. CAHDS Referrer Information. CAHDS Information Brochure. Further information on CAHDS is available on the:

Steps	Additional Information
can be sent by fax, email, e-referrals or post. Referrers may provide the 'CAHDS Information Brochure' to families at the time of referral.	• CAMHS Specialised services and day programs webpage • CAHDS hub page.
2. Triage The CAHDS CNS Triage assesses referrals against the entry criteria. Once CAHDS administration places the referral on PSOLIS the referral is forwarded to the CAHDS CNS Triage. CAHDS CNS triage follows up with the referrer to obtain previous reports and paediatric letters. Once these are received (within a 4 week timeline) CAHDS CNS Triage brings the referral to CAHDS Intake meeting, which is attended by CAHDS CNS Triage, Head of Service and Service Manager. If a referral is accepted CAHDS CNS Triage contacts the family via telephone to arrange an appointment to discuss the referral and CAHDS process/service. This appointment is usually via telephone and is made at a date and time agreed with the family. CAHDS Developmental Questionnaires are forwarded via email to the parents to complete and return prior to the appointment. CAHDS CNS Triage provide the family with their initial psychosocial appointment with the allocated CAHDS case coordinator. Referrer is copied into the email sent to family confirming psychosocial appointments. If the referral is not accepted: the CNS informs the family and discusses alternative services and	Entry Criteria Under the age of 18 years and diagnosed to have ADHD. Currently prescribed /being considered for the prescription of medication to treat ADHD. The presence or suspicion of comorbid disorders that are resulting in significant complexity and functional impairment in one or more areas of the child and family's life. Screening Questionnaires and CAMHS information are emailed to family prior to Triage appointment. • CAHDS Developmental Questionnaires ○ CAHDS Child Development Questionnaire. ○ CAHDS Mother Respondent Birth and Development Questionnaire. ○ CAHDS Father Respondent Birth and Development Questionnaire • CAMHS Consumer representative flyer • Welcome to CAMHS • Your rights information sheet for families.

Steps	Additional Information
recommendations with the family. The referrer is notified of recommendations made and any suitable alternative service/s. A copy of this letter is also sent to the family.	
3. Assessment All families attend separate parent and child psychosocial assessments with their CAHDS case coordinator. At this assessment informed consent and exchange of information with relevant services (e.g. schools, Child Development Services, private clinicians) are discussed and obtained and screening questionnaires are provided to the family to complete and return to CAHDS. Following this assessment, the case coordinator presents the information obtained during the psychosocial assessment to the team at the next MDT review meeting. At this meeting the team identifies which disciplines will become involved in the assessment process either directly assessing and/or reviewing previous assessments/reports. On occasion, due to clinical reasons, it may be decided by the team that CAHDS is not the most appropriate service for the child at the current point in time and the referral is closed. The case coordinator then informs the family of the outcome of the MDT review meeting. If the referral is closed, alternate/ more appropriate services are recommended to the family. This information is also provided to the referrer. Discipline specific and/or further assessment appointments are arranged by administration via telephone and then confirmed via email or letter.	Screening questionnaires given at psychosocial appointment: • Parent, and/or self-report versions (depending on child's age) of: ○ Connors 4th Edition ○ Child Behaviour Checklist (Achenbach) ○ Behaviour Rating Inventory of Executive Function Second edition(BRIEF2) ○ Strengths and Difficulties Questionnaire (SDQ) ○ Sleep disturbance Scale for children ○ Parenting Stress Index 4th edition (PSI4) ○ WEISS symptom record II ○ WEISS functional impairment rating scale – parent report (WFIRS-P) ○ SNAP-IV – Teacher and Parent Rating Scale. ○ Social Communication Questionnaire (SCQ) ○ The Childhood Autism Spectrum Test (CAST) See: CAMHS Multidisciplinary Team Clinical Reviews Policy Teacher versions: ○ Connors 4th Edition ○ Teacher Report Form (Achenbach) ○ Behaviour Rating Inventory of Executive Function Second edition(BRIEF2) ○ WEISS symptom record II ○ WEISS functional impairment rating scale – parent report (WFIRS-P) ○ SNAP-IV – Teacher and Parent Rating Scale. ○ CADDRA Teacher Assessment Form ○ CAHDS Teacher Questionnaire

Steps	Additional Information
Discipline specific assessments relevant for the child, as indicated following the psychosocial assessment, are undertaken. School information is requested via the CAHDS school liaison psychologist or case co-ordinator. School questionnaires are forwarded to the school for the teacher to complete and return to CAHDS. Following the completion of CAHDS assessments the relevant CAHDS team members attend a Formulation Meeting to discuss assessment findings and recommendations. These are recorded in the CAHDS MDT Report which is provided to the family, referrer and school (if appropriate and with parental consent).	○ CAHDS School Psychologist Questionnaire
4. Feedback and Exit Following the assessment, copies of CAHDS assessment reports are provided via mail to the family and referrer, prior to the Family Feedback Session. A face to face Family Feedback Session is offered to the family to discuss the CAHDS assessment findings and recommendations. With parental consent, a School Feedback Session is offered to the school (with the option for the family to attend). This is completed via telehealth. Further feedback to referrer is provided, if necessary.	

Consultation Pathway for other CAMHS Teams

CAHDS also provides consultation service to other CAMHS Teams (see [Request for CAHDS Consultation for Active CAHS CAMHS Clients Information Sheet](#)). Referrals are accepted from other CAMHS teams for specialist CAHDS allied health

assessments, to clarify the presence of any neurodevelopmental co-morbidities and assist the CAMHS team in formulation and treatment planning of complex presentations. All children and young people referred under this pathway must have attentional difficulties as one of their presenting features. A confirmed diagnosis of ADHD is not a pre-requisite to access this pathway. Children and young people accessing this pathway are not usually seen by the CAHDS Psychiatrist as they would access the Psychiatrist in their respective CAMHS team. To access this pathway, CAMHS clinicians are required to complete the Consultation referral form (see [CAHDS Consultation Referral Form for Active CAHS CAMHS Clients](#)) and attach the Standardised Initial Assessment document.

Related CAHS internal policies, procedures and guidelines

[CAMHS Consent Guideline](#)

[CAMHS Multidisciplinary Team Clinical Reviews Policy](#)

This document can be made available in alternative formats on request.

File Path:	CD/25/90413		
Document Owner:	Director ICAMHS South		
Reviewer / Team:	Complex Attention and Hyperactivity Disorders Team		
Date First Issued:	29/07/2014	Last Reviewed:	August 2025
Amendment Dates:		Next Review Date:	10/09/2028
Approved by:	CAMHS Director Clinical Services	Date:	24/09/2025
Endorsed by:	CAMHS Clinical Governance Executive Committee	Date:	01/10/2025
Aboriginal Impact Statement and Declaration (ISD)		Date ISD approved:	
Standards Applicable:	NSQHS Standards:  Child Safe Principles: 2, 3, 4, 5, 6, 7		
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 Healthy kids, healthy communities CompassionExcellenceCollaborationAccountabilityEquityRespect Neonatology Community Health Mental Health Perth Children's Hospital			